PARTNER SPONSORSHIP · DRAFT FOR DISCUSSION

Decision Advantage

How much hospital delay is a decision waiting, and how much is a missing bed?

November 2026 · Prepared by [Your name]

Delays are counted, but their cause is not

562

patients ready to leave hospital were still in beds at midnight on 1 Nov 2026. National KPI: 300. Verified.

+16.8%

delayed-transfer bed days, Q1 2026 vs Q1 2025 (42,123 vs 36,078). Plausible: TD citing HSE data.

217 of 526

were recorded as waiting for residential care on 6 July. The category does not show a missing bed or a decision waiting. Verified.

Hypothesis: some delay is a decision waiting, not a missing bed. Nobody publishes how much.

A one-page decision contract measures it

Element	What it records
Trigger	An agreed threshold
Accountable owner	A named person and deputy decide; the system only routes
Deadline	Acknowledge, then decide or escalate, within agreed times
Escalation path	Who is told if no decision is made or the action is infeasible
Reason	Evidence considered, decision taken, reason for any exception
Feasibility class	Decision delayed, action infeasible, or action done without expected result
Outcome	Whether the agreed result occurred, without assuming cause

Non-clinical, read-only, no patient-level data. People make every decision.

Realistic impact: about one bed at a large site

Share blocked by a pending decision (unknown)	Avoidable bed days / 90 days	10-15% removed	Beds freed
6%	228	23-34	0.3-0.4
15%	571	57-86	0.6-1.0
24%	913	91-137	1.0-1.5

Illustrative, at Galway's 2025 run rate (about 3,800 delayed-transfer bed days per 90 days; preliminary HSE data, to verify).

Not claimed

A 10-15% cut in all delayed transfers or in trolleys. The evidence does not support it.

First prize

Knowing how much delay is decisions, and evidence on blocked feasible transfers for capacity planning.

Why EY

The 2018 review

EY conducted the independent review of delayed discharges for the Department of Health. Data limits prevented ranking the causes. This diagnostic measures that gap.

Digital twin work

EY’s article says to anchor on one specific decision. A twin shows what a breach implies; the contract records what was decided and what happened.

Shared Care Record

EY is building it for the HSE. A relationship, and an independence check before any HSE contact.

The ask: two steps, one gate

Step 1: 90-minute workshop

- Two hours of preparation; synthetic or published data
- Life Sciences, AI & Data / twin, health operations, risk [names to confirm]
- Must end with one candidate engagement, or a stop

Step 2: 12-week EY-led assessment

- Only if Step 1 passes
- Interviews, workflow map, baseline, read-only shadow mode, stop or go
- Partner time [__] hours; team [__] people

Gates before week 1: named sponsor · client route · resource approval · IP and independence clearance

Go and stop criteria

Go if

- Four or more interviewees at two or more sites describe a recent case with no owner, reason or outcome recorded
- A site will share aggregate data
- Data protection sees a route without patient-level data

Stop if

- An existing record already meets the contract
- Capacity is the binding constraint
- No budget holder emerges
- The work needs patient-level data

Decision for sponsors: Step 1 now; Step 2 only if it passes.

Complementary to EY’s digital twin work

What twins already do

- Anchor work in a specific decision
- Test changes before implementation
- Combine data with clinical expertise

What we would test

- Who owns the response after the insight
- A record of rationale, escalation and outcome
- Outcomes reviewed against twin assumptions (a hypothesis)

We do not claim that twins lack decision governance. We test whether a gap exists.

How we got here: evidence-led narrowing

Stage	What it showed	Not yet validated
Thought leadership	A hypothesis about delayed decisions in hospital operations	That the problem exists at a real site
Build Week prototype	The decision lifecycle works on synthetic data	Performance, adoption, client value
HIHI feedback	Crowded market; differentiation and a real gap unproven	Differentiation
Narrowing	One decision contract; lead case moved to specialist clinic capacity to match EY's published work	Client demand
Today	A proposal and a workshop request	A client workflow, buyer, data route or commercial case

First use case: a specialist clinic-capacity decision

Similar to the oHCM example in EY’s article. When a pre-agreed demand or capacity condition occurs: who decides whether to activate an approved response, what blocks it if infeasible, and what happens next?

Bounded

One decision, one pathway, one measure.

Non-clinical

Operational response only; clinical judgement stays with clinicians.

Planning-led

Matches the article’s focus on testing choices in advance.

The lead case moved from ED boarding to match EY’s published work. That improves alignment; it does not validate demand. ED boarding stays a possible later case.